



“Fighting Off a Monsoon of Hate”: Stories from the Frontlines of Gender-Affirming Care

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Background and Aims

- Gender-affirming and transition-related care (GATRC) is evidence-based, medically necessary treatment that improves the health and well-being of transgender and gender diverse (TGD) individuals.
- Despite broad clinical consensus on its safety and efficacy, U.S. legislation restricting or criminalizing GATRC has risen sharply.
- Between 2018 and 2022, proposed bans on GATRC for minors increased more than tenfold, with some states imposing civil penalties, loss of licensure, or imprisonment for providers.
- While patient harms are well-documented, less is known about the impacts on the providers delivering this care.
- The Measuring TAPE (Trans-Affirming Providers’ Experiences) Study aims to address this gap by examining the professional and personal impacts of anti-trans legislation on providers of GATRC.

Methods

- Participants: $N = 124$ (qualitative analytic sample $n = 109$) U.S. medical and mental health providers offering gender-affirming care (M age = 39.2, SD = 10.2).
- Data were collected September 2024 – March 2025 via snowball sampling and targeted outreach.
- A mixed-methods, cross-sectional online survey included four open-ended questions.
- Questions explored the consequences of recent legislation, impacts on professional relationships, media and political pressures, and effects on patients.
- Responses were coded for common experiences, events, and emotions.
- Superordinate and subordinate themes were identified and supported with participant quotes.

Results

Patients suffer as they navigate a restrictive healthcare environment

- Psychological and existential distress
- Political preoccupation

“My clients experience it as an existential threat to their very existence. Low self-esteem, limited appearing horizons, generalized distrust of government, education, and the medical/psychological and other helping professions are all examples I am seeing daily. Pre-Biden dropping out a generalized hopelessness, a resigned sense of doom dominated the vibe my clients were expressing about the political future. Most picture being rounded up in camps and forced to detransition.”

Providers’ distress as primary and secondary trauma

- Primary trauma
- Secondary trauma
- Environmental stressors

“I haven’t directly been impacted, but my clinical work has been impacted because of the aforementioned increase in distress with my transgender and gender non-conforming [clients]. It can feel hopeless as a provider: as though my clients and [I] are fighting off a monsoon of hate directed at them.”

Disruption to work environments and healthcare systems

- Sanctuary states overloaded
- Conflict among colleagues
- Threats to provision of care

“I live and work in state that is supportive of GATRC, and has shield laws for providers and patients[.] I struggle [to] work with patients to get records and be able to communicate with their home therapists or primary care team—or prior GATRC providers, without doing harm; making sure patients can get labs and medications safely and consistently without fear. Each visit takes longer and requires more support staff and supportive counseling to patients and families—and these are the lucky ones, the ones I get to see.”

The danger of politically-motivated misinformation

- Misinformation is pervasive
- Misinformation and rhetoric fuel violence

“As a care team, we are fearful of our own safety from targeting and doxing. Luckily, we haven’t had an incident...yet. But the disgusting political ads treating trans folks like monsters and criminals [have] emboldened more hate speech and violence toward individuals and care providers.”

Elevated risk and vulnerability among youth

- Higher risks for youth-serving providers
- Existential dread & hopelessness

“I work with [predominantly] queer clients, and I do a significant amount of work with minors (middle and high school age). I have seen a horrific decline in my queer/gender diverse adolescents’ mental health after laws were passed that have made it harder for schools and teachers to be accommodating without backlash. Iowa has been a hellscape for queer folks and not only am I experiencing it myself, I am seeing it firsthand in my therapy office.”

Hope and resistance

- Colleagues inspire and support each other
- Providers “willing to take the heat” for their patients

“I also have some patients and families who are resolute and will not back down, and I both draw strength from them and project my own power to reassure and encourage my patients and their parents. This is the moment all my training and work has been leading me to; history will not look kindly on the bigots, but I refuse to be a good person who does nothing in the moment.”

Discussion

- Thematic analysis revealed themes of fear and precarity, moral and existential distress, resistance and resilience, and overall negative impacts on healthcare systems.
- Respondents described an ongoing wave of misinformation that emboldens transphobic rhetoric, encourages open hostility toward providers, and perpetuates violent policymaking, contributing to direct and secondary trauma for providers and increased vulnerability among patients, especially youth.
- These pressures carry broad systemic and interpersonal consequences, undermining providers’ ability to deliver care, support their patients, and maintain professional relationships, while also straining healthcare systems.
- Despite these challenges, participants reported resilience, strengthened collegial support, and care innovation in response to escalating pressures.

Limitations and Future Directions

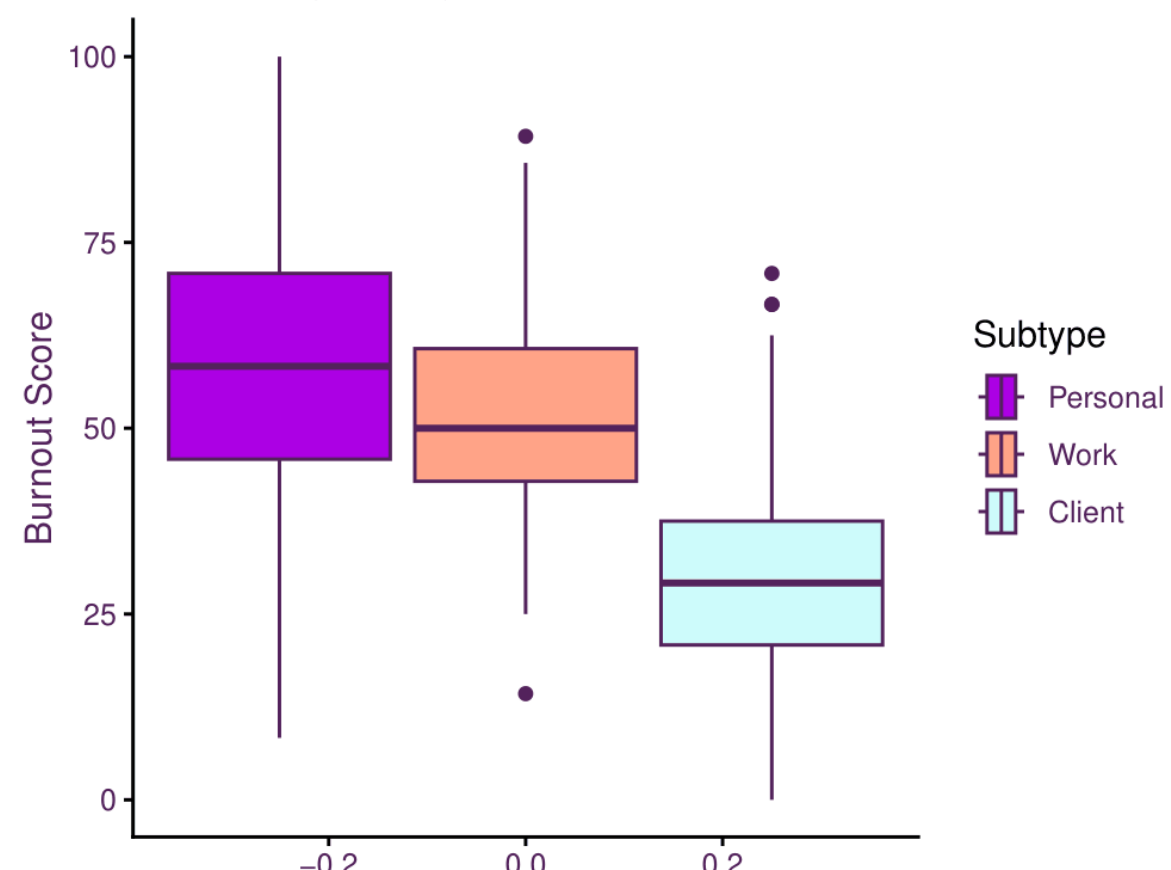
- Limitations include small sample size, predominantly white participants (85.6%), and cross-sectional design during a period of significant sociopolitical change.
- Future research should continue to explore narratives from health providers affected by anti-trans healthcare legislation. Mixed methods are strongly encouraged to strengthen advocacy.

Acknowledgments

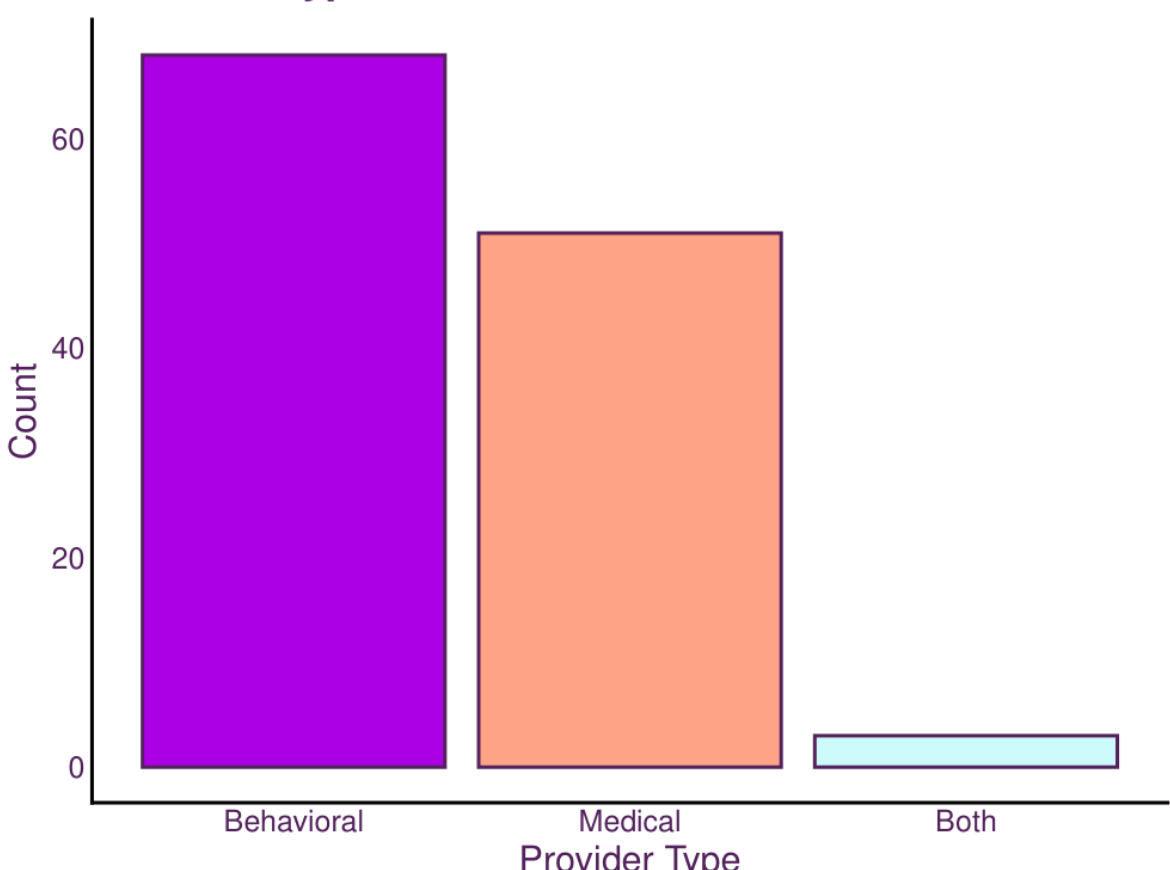
- The FLAIR team: Shannon Collins, Sal Daña, Dumayi Gutierrez, Anthony Jackson, Noam Kazis Taylor, Andy Price, Courtney Rago, Theo Stein, Faith Wood, and Kalen Zeiger.
- The Gender & Family Project at the Ackerman Institute.
- Most of all, we thank our participants for sharing their perspectives and experiences.

Sample

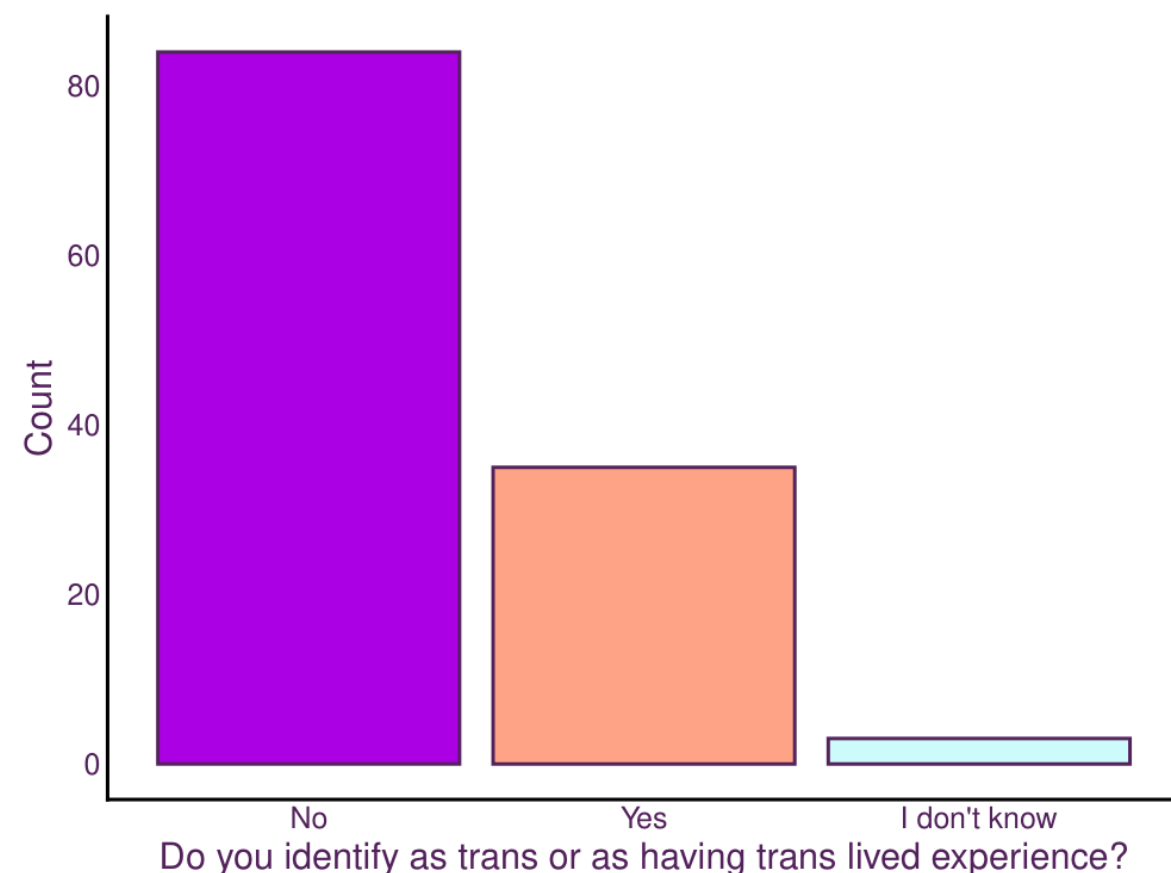
Personal, Work, and Client Burnout Scores



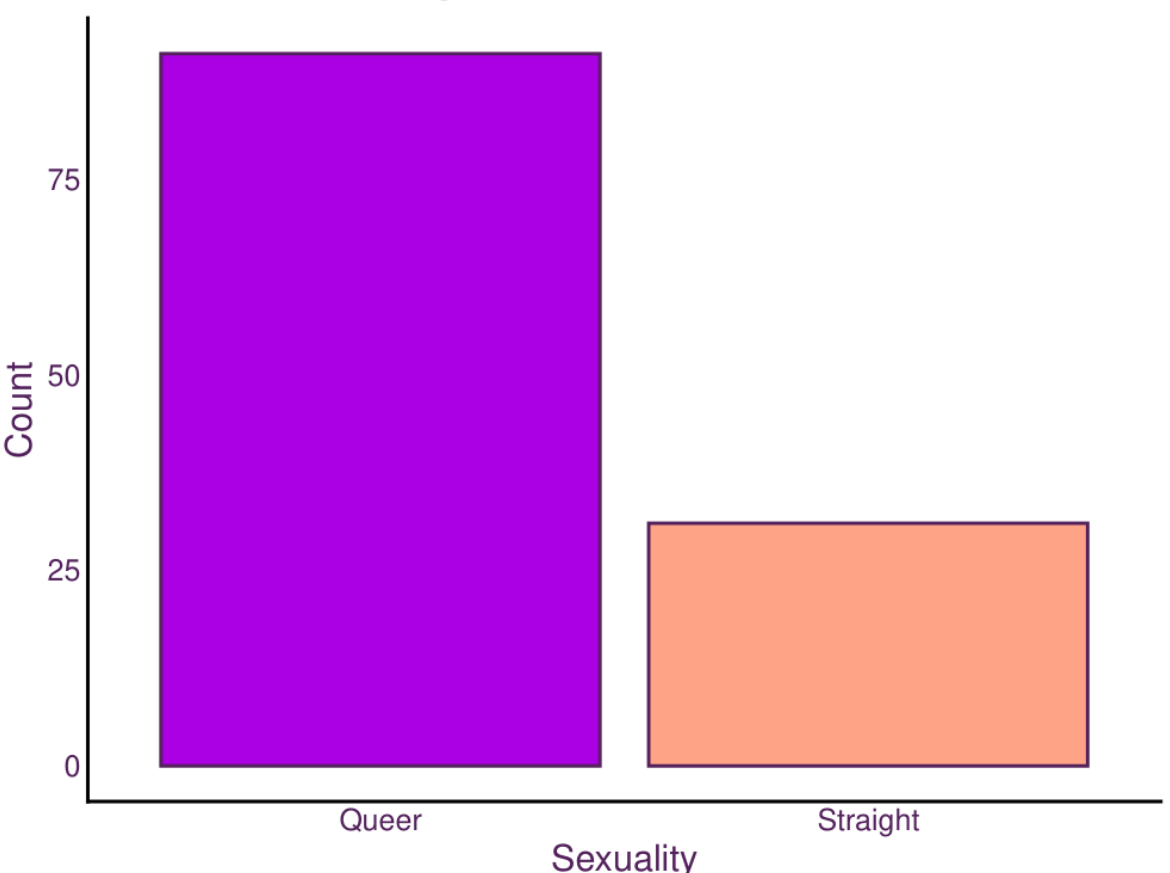
Provider Type



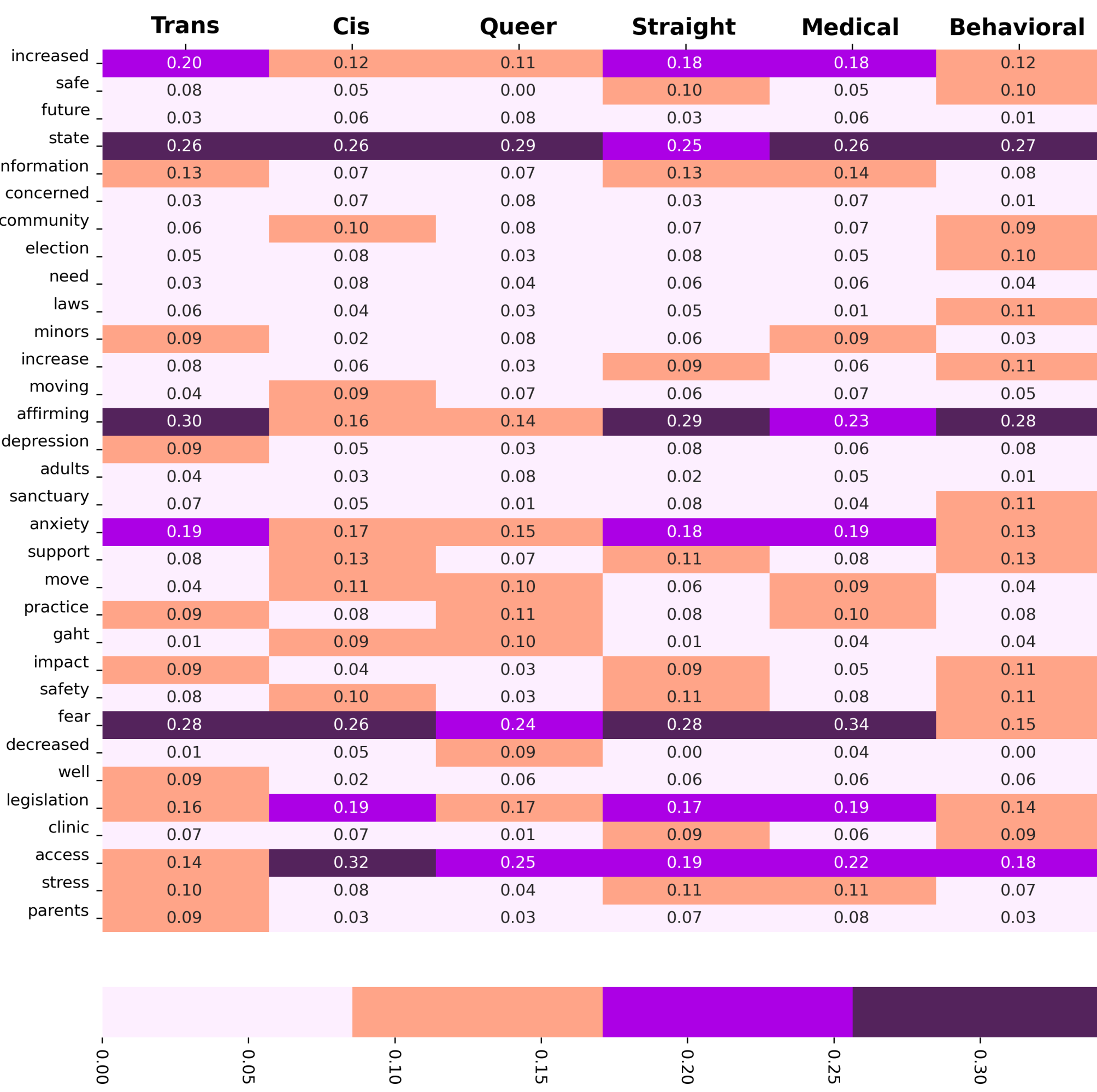
Trans Providers



Provider Sexuality



TF-IDF Matrix



FLAIR Lab



Gender &
Family Project